

OREGON STATE HOSPITAL

POLICY

SECTION 6: Patient Care **POLICY: 6.035**

SUBJECT: Medication Reconciliation

POINT PERSON: Chief of Psychiatry

APPROVED: Sara Walker, MD  **DATE: JULY 25, 2024**
Interim Superintendent

SELECT ONE: ☐ New policy ☐ Minor/technical revision of existing policy
☒ Reaffirmation of existing policy ☐ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. Medication reconciliation is an interdisciplinary process designed to maintain an accurate list of an Oregon State Hospital (OSH) patient's medications and decrease adverse drug events, thereby reducing the risk of medication-related injury to patients. This policy and its attachments establish guidelines and steps that must occur for medication reconciliation.
- B. This policy applies to all staff who are responsible for the ordering, dispensing, or administration of patient medication.

II. POLICY

- A. Every patient at Oregon State Hospital (OSH) must have all medications reconciled upon admission, transfer, and discharge according to the appropriate procedures in Procedures A.
- B. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
- C. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

- A. “High-risk, high-alert medications” include those medications involved in a high percentage of errors or sentinel events, or carry a higher risk for abuse, error, or other adverse outcomes. OSH identifies the following medications as High Risk, High Alert – insulin, anticoagulants, clozapine, lithium, oxcarbazepine, and metformin.
- B. “Medication reconciliation” means the formal process of creating a complete and accurate list of each patient’s current medication at the time of admission, transfer, or discharge.
- C. “Staff” includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).

IV. PROCEDURES

Procedures A Medication Reconciliation Procedures

V. RELATED OSH POLICIES AND PROTOCOLS

6.001 Patient Transfers

6.013 Discharge and Conditional Release Planning

6.055 Medication Management

6.058 Admissions

Medical Department protocol 1.002 Physician Orders

Medical Department protocol 4.001 Antipsychotic Polypharmacy

Medical Department protocol 5.001 Informed Consent

Pharmacy Department protocol 1.002 Adverse Drug Reaction Reporting

Pharmacy Department protocol 1.005 Anticoagulation Therapy

Pharmacy Department protocol 1.007 Automatic Stop Orders

Pharmacy Department protocol 3.002 Clozapine Management System

Pharmacy Department protocol 13.002 Medications and Nutritional/Herbal Supplements

VI. REFERENCES

Joint Commission Resources, Inc. (2018). *The joint commission comprehensive accreditation manual for hospitals*. MM. 03.01.05, 04.01.01, 05.01.01, 02.01.01. Oakbrook Terrace, IL: Author.

Oregon Administrative Rules §§ 309-114-010 — 309-114-015

Oregon Administrative Rule § 309-031-0010.

Oregon Administrative Rules §§ 309-033-0270 – 309-033-0320.

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Oregon Administrative Rule § 309-091-0025.

Oregon Revised Statute § 179.473.

Oregon Revised Statute § 419C.530.

Oregon Revised Statute § 420.505.

Oregon Revised Statute § 426.060.